Operating Department Standard Of Practice YG

ODSOP No: 53 Peri-operative Hypothermia.

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Aim: To prevent, recognise and treat inadvertent hypothermia in the surgical patient and aim to maintain the patients normothermia throughout the perioperative pathway

Scope

- All theatre staff

Responsibilities

- Theatre Manager

Will ensure principles of patient safety and comfort are promoted within the department.
Will ensure equipment necessary to recognise, treat and prevent inadvertent hypothermia is available and fit for purpose and that staff receive appropriate training in their use.

- Team Leaders

Will ensure members of their team are well informed as to the risks associated with inadvertent hypothermia, are aware of this ODSOP and understand the actions required to promote patient normothermia and to prevent, recognise and treat patients' inadvertent hypothermia.

- Theatre staff

Will maintain the standards of practice of patient care required to promote patient normothermia, and to prevent, recognise or treat inadvertent hypothermia.

Hypothermia is defined as a core temperature less than 36°C (96.8°F)
(NICE Guideline No:65 April 2008)

Negative consequences associated with peri-operative hypothermia include the following: -

- Patient discomfort from shivering
- Untoward cardiac events
- Impaired platelet function leading to increase of peri-operative bleeding
- Altered drug metabolism
- Impaired wound healing with increased susceptibility to infection

Pre-operative: -

- Patients are to be kept comfortably warm (36.5°C - 37.5°C. NICE Guideline 65) on the ward prior to collection for transportation to theatre
- Ensure patients are kept comfortably warm and well covered when being transferred to theatres as per ODSOP No.21. Transporting Patients to and from the Operating Theatre.doc.
- Consider the use of additional blankets, hats and socks, dependant on the patients' individual needs and need for preoperative warming.
- At theatre reception, the patient is asked if they are feeling comfortably as routine.
- Warming measures may need to be initiated prior to anaesthesia if the patients temperature is below 36°C (96.8°F)

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Immediate Pre-operative Stage [anaesthetic room]

- Consider the use of a temperature probe as part of monitoring.
- Consider the use of additional blankets or forced air appliances for patients undergoing regional block or invasive monitoring techniques prior to their surgery.
- If the patient requires Intra-venous therapy a fluid warming device should be considered, particularly if more than 500ml or more is to be used.
- The Hotline fluid warmer should be the first choice, using the Level 1 only if major blood loss is anticipated].

Intra-operative: -

- The aim of warming devices is to maintain patient temperature of at least 36.5°C during the surgical procedure (NICE Guideline No 65)
- Warming methods such as Bair huggers, fluid warmers or a combination of both] should be considered for
    1. Surgery expected to exceed 30 minutes' which includes the time spent in the anaesthetic room
    2. Fluids' therapy of volume 500ml or more as per NICE guidelines No 65
    3. Laparoscopic Surgery
    4. Intra abdominal / pelvic surgery
    5. TURP / TURBT when large volumes of irrigation fluids are used.
    6. All children and elderly patients.
    7. When blood and blood products are used.

Other considerations: -

- Maintain an appropriate ambient temperature throughout the procedure
- All patients should be kept covered as much as possible exposing only the surgical site,
- Time taken between 'prepping and draping' is to be kept to a minimum
- Keep the patient dry e.g. provide clean dry draw sheets and gowns when appropriate and prevent surgical drapes from becoming wet.
- Irrigation fluids should be kept in a warming cabinet 38-40°C prior to administration.
- Avoid the use of an I.V. extension set when using fluid warmers
- Patients' temperature should be measured and documented before anaesthesia and every 30 minutes until the end of surgery.

Post-operative

- Ensure the patient is suitably covered with blankets immediately after surgical drapes are removed.
- Routine observations must include temperature monitoring.
- Maintain or increase patient temperature through use of Bair Hugger and/or fluid warming device as applicable so as to meet the Recovery discharge criteria of a core temperature of >36° C. as per ODSOP No.14 Recovering Patients Following Anaesthesia. doc
- Patients should be feeling comfortably warm prior to transfer to the ward

References: -

- N.I.C.E Guideline No 65 The Management of Inadvertent Perioperative Hypothermia in Adults April 2008.
- AfPP Standards and Recommendations for safe Perioperative Practice June 2011
- AFPP QUAD Document 2006
- ASPAN Evidence based clinical practice guideline for promotion of perioperative normothermia. 2nd edition 2010